

ROUTINE POSTNATAL CARE

Give urgent attention to the postnatal patient (within 6 weeks of delivery) with any of:

- Heavy bleeding (soaks pad in < 5 minutes): **postpartum haemorrhage** likely
- Fitting or just had a fit up to 1 week postpartum: treat as **eclampsia** → 159.
- Unwell and temperature $\geq 38^{\circ}\text{C}$
- Perineal tear extending to anus or rectum
- BP < 90/60
- Pulse ≥ 100
- Hb < 6
- Pallor with respiratory rate ≥ 30 , dizzy, faint or chest pain

Manage and refer urgently:

- If BP < 90/60, give **sodium chloride 0.9%** 1L IV rapidly, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- If **postpartum haemorrhage** likely: call for help, this is a life-threatening condition and requires immediate referral. Manage urgently:
 - Massage uterus, remove clots from vagina and empty bladder (with catheter if needed).
 - Whilst setting up IV, give **oxytocin** 10units IM if not already given after baby delivered. Give **oxytocin** 20units in 1L **sodium chloride 0.9%** at 250mL/hour IV in a 2nd IV line.
 - Ensure placenta is delivered. If controlled cord traction fails, try manual delivery.
 - If uterus still soft after this:
 - Give **ergometrine** 0.5mg IM or **oxytocin/ergometrine** 5units/0.5mg (1mL) IM and continuously massage uterus. Avoid if eclampsia, pre-eclampsia, known hypertension or heart disease unless bleeding is life-threatening. Repeat after 10–15 minutes if no response to 1st dose, while arranging referral.
 - Only if oxytocin and oxytocin/ergometrine unavailable, give **misoprostol** 600mcg rectally or sublingually.
 - Repair any bleeding tears.
 - If still bleeding heavily, insert balloon catheter¹ into uterus, inflate with 400–500mL of saline, clamp catheter and pack vagina with swabs to prevent expulsion.
 - Apply bimanual compression² during transfer.
- If unwell and temperature $\geq 38^{\circ}\text{C}$: give **ceftriaxone** 1g IV³/IM. If painful abdomen or foul-smelling vaginal discharge, also give **metronidazole** 400mg orally.

Assess the mother and her baby 6 hours, 6 days, and 6 weeks after delivery.

Assess	When to assess	Note
Symptoms	Every visit	Manage mother's symptoms as on symptom page. Manage baby's symptoms with IMCI guide.
Mental health	Every visit	• In the past 2 weeks, has patient: 1) been unable to stop worrying or thinking too much 2) felt down, depressed, hopeless? If yes to any → 143. • In the past 2 weeks, has patient had thoughts or plans to harm herself? If yes → 83.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ⁴ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142.
Family planning	Every visit	Assess patient's contraceptive needs → 154. Ideally, insert IUD within 48 hours of delivery if no contraindications ⁵ or, insert subdermal implant or start injectable or POP at any stage after delivery, or offer tubal ligation if appropriate. Avoid COC pill for 6 weeks after delivery and for 6 months if breastfeeding.
Infant feeding	Every visit	If breastfeeding: check for breast problems → 43. Check that baby latches well. If formula feeding: ensure correct mixing of formula and water.
Baby	Every visit	• Assess and manage the baby according to the IMCI guide. • Ensure baby gets immunisations at birth and 6 weeks. If mother known to have TB, hepatitis B or syphilis, prevent infections in the newborn → 167.
Psychosocial risk	Every visit	Help access support especially if at risk of mental health problem: patient not interacting with baby, difficult life event in last year, unhappy about pregnancy, absent/unsupportive partner, violence at home, abused as a child, no social/family support, previous depression/anxiety, < 20 years, no money for food, patient is a refugee or has HIV.
Abdomen and perineum	Every visit	• If painful abdomen or foul-smelling vaginal discharge, refer/discuss same day. • If perineal or abdominal wound: check healing. Advise salt baths twice daily in warm water for perineal wounds. If red/warm/painful/swollen/foul-smell/oozing pus, discuss/refer.
BP	Every visit	Check BP. If BP $\geq 140/90$, recheck after 1 hour rest. If BP still $\geq 140/90$ → 132, unless ≤ 1 week postpartum: discuss same day.
BMI	Every visit	Mother's BMI = weight (kg) $\div$ height (m) $\div$ height (m). If < 18.5, arrange nutritional support.

Continue to assess the postnatal patient and baby → 165.

¹If balloon catheter unavailable, make condom catheter: slip open condom over large Foley's catheter and tie with string at the base. ²Bimanual compression: insert clenched fist into vagina, with back of hand posteriorly. Place other hand on abdomen behind uterus and squeeze uterus firmly between two hands. ³Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ⁴One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁵Avoid IUD if: chorioamnionitis, rupture of membranes for > 18 hours or postpartum haemorrhage.